



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°122/MVEBDB/09/13/2026

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HIGHLIGHTS (last 24 hours)



In the past 24 hours, 58 new confirmed cases of Ebola Virus Disease (EVD) have been reported, including 23 community deaths were recorded. These cases originated from the provinces of Ituri (44 cases), North Kivu (12 cases), and Haut-Uélé (2 cases). In addition, 12 deaths within the Ebola Treatment Center (ETC) were also recorded.

✓ No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 7,258	ACCUMULATIVE DEATH CONFIRMED 3,510	LETHALITY 48.4%	PATIENTS IN ISOLATION/CTE 905	ACCUMULATIVE HEALED 1,726	TRACKING RATES CONTACTS (Of the Day) 78.6%
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Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



62 out of 167
health zones affected
(37.1%)

Summary of key points from the last 24 hours

During the course of the day on September 13, 2026, **58 new confirmed cases** of Ebola Virus Disease (EVD), with **23 community deaths** recorded. These confirmed cases are from the provinces of **Ituri (44 cases), North Kivu (12 cases), Haut-Uélé (2 cases), and Tshopo (0 cases)**. In addition, **12 deaths within Ebola Treatment Centers (ETCs)** were reported, including 6 in Ituri and 6 in North Kivu, bringing the total number of deaths for the day to **35**.

However, for more or less 42 days, there has been an **absence of notification of new confirmed cases** in 7 health zones of the provinces of Ituri (Adja, Kambala and Mahagi), North Kivu (Goma), South Kivu (Miti-Murhesa) and Tshopo (Lubunga and Wanierukula).

In total, 14 patients were declared cured after presenting two negative results for Ebola virus disease, respectively **12 in Ituri and 2 in North Kivu**.

Since the start of the epidemic, the DRC has recorded a total of **7,258 confirmed cases, including 3,510 deaths**, representing a case fatality rate of **48.4 %**. The main hotspot remains the Ituri province, which accounts for **78.0%** of the confirmed cases reported in the 7 affected provinces and also records **75.9%** of new confirmed cases.

No new health zones affected in the last 24 hours. DRC

continues to have **62 health zones out of 167 (37.1%)** affected and **7 provinces affected**. Ituri has 28 health zones affected out of 36, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 13, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	44	5659	2,583	45.6%	28/36 (77.8%)
North Kivu	12	1281	797	62.2%	16/34 (47.0%)
Haut-Uélé	2	281	115	40.9%	6/13 (46.1%)
Tshopo	0	28	10	35.7%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	5	3	60.0%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	58	7,258	3,510	48.4%	62/167 (37.1%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 13, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5659	2583	45.6%	44	18	6	24
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	179	39	21.8%				0
Boga	2	2	100.0%				0
Bunia	1583	505	31.9%	19	10		10
Damascus	39	9	23.1%				0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	135	88	65.2%	3	0		0
Lita	262	130	49.6%	3	3		3
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	47	25	53.2%				0
Mangala	313	146	46.6%	1	0		0
Mongbwalu	663	303	45.7%	5	5		5
Nia-Nia	239	120	50.2%	2	0		0
Nizi	732	273	37.3%	3	0		0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0

Rwampara	1019	349	34.2%	6	0		0
Tchomia	71	25	35.2%	2	0		0
To ventilate *	N / A	442	N / A			6	6
North Kivu	1281	797	62.2%	12	5	6	11
Blessed	228	166	72.8%	6	2	2	4
Good	17	7	41.2%	1	0		0
Butembo	239	157	65.7%	3	1	4	5
Goma	1	0	0.0%				0
Kalunguta	30	12	40.0%				0
Katwa	523	332	63.5%				0
Kayna	1	0	0.0%				
Kyondo	30	19	63.3%	1	1		1
Lubero	15	7	46.7%				0
Mabalako	19	9	47.4%				0
Manguredjipa	8	3	37.5%				0
Masereka	20	8	40.0%				0
Musienene	116	63	54.3%	1	1		1
Mutwanga	6	2	33.3%				0
Oicha	11	7	63.6%				0
Vuhovi	17	5	29.4%				0
Haut-Uélé	281	115	40.9%	2	0	0	0
Boma	32	14	43.8%				0
Mangbetu							
Gombari	3	1	33.3%				0
Isiro	97	41	42.3%	1	0	0	0
Pawa	58	25	43.1%	1	0	0	0
Rungu	2	1	50.0%				0
Wamba	89	33	37.1%				0
Tshopo	28	10	35.7%	0	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	4	2	50.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas-Uélé	5	3	60.0%	0	0	0	0
Buta	1	1	100.00%				0
Viadana	1	0	0.00%				0
Ganga	3	2	66.67%				0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7258	3510	48.4%	58	23	12	35

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province, as of September 13, 2026

Table 1: Status of alerts received by province, as of September 23, 2020									
DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	17	3	20	9	3	8	0	12	9
Bas-Uélé	20	1	21	3	1	17	0	4	0
Ituri	748	54	802	99	54	501	0	153	82
North Kivu	879	45	924	126	44	702	1	170	94
South Kivu	5	0	5	0	0	5	0	0	0
South Ubangi	3	0	3	3	0	0	0	3	0
Tshopo	189	1	190	8	1	181	0	9	4
Total General	1861	104	1965	248	103	1,414	1	351	189

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

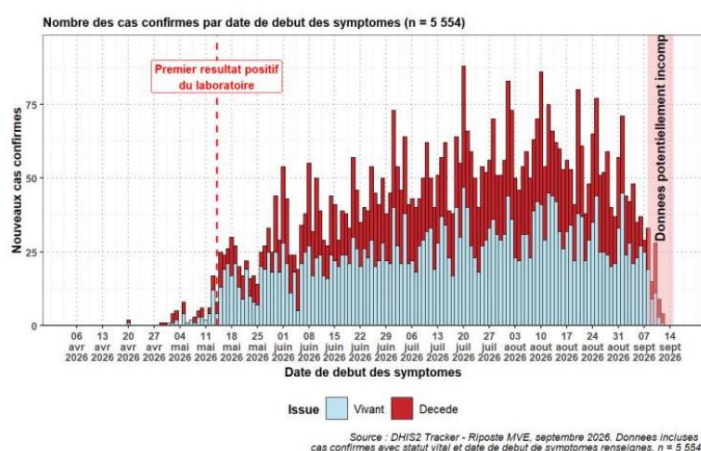
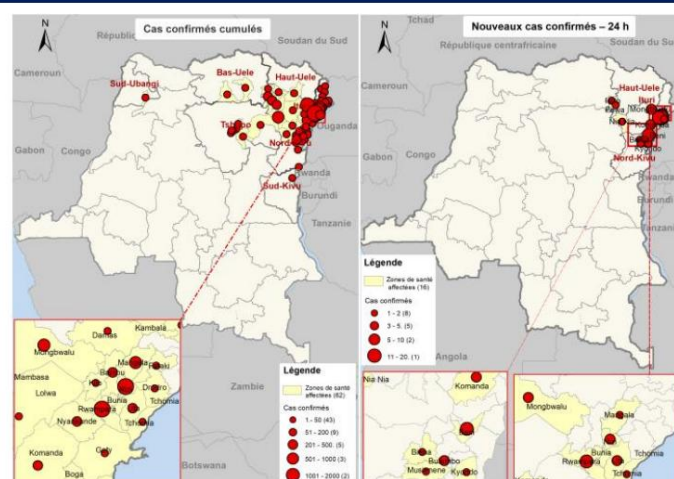


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Across the seven affected provinces, **1,965 alerts** were reported on September 13, 2026, of which 1,766 (89.9%) were verified. Of these, **351 alerts were validated as suspected cases**. All of these were investigated (100%), and **189 were referred to the CT/CTE** (Table 3).

Regarding the contact tracing rate, it stands at 78.6% (21,935/27,905), down from the previous day (88.0%) and well below the threshold set at 85% due to poor data completeness in several health zones of the North Kivu provinces, Haut-Uélé; 87.6% in Ituri (12,561/14,346), 83.2% in Bas-Uélé (149/179), 77.9% in the

Tshopo (218/280), 69.2% in North Kivu (8,146/11,774), 65.7% in Haut-Uélé (845/1,287) and 41.0% in South Ubangi (16/39).

1.1.3. Challenges

Contact tracing is suboptimal in 3 out of 5 provinces with contacts currently being monitored, with a significant decline in the last 24 hours in North Kivu province.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

One (1) alert was notified to the Irumu PoC (Nyankunde Health Zone) in Ituri. It concerns a 51-year-old man traveling from Mambasa to Bunia. The alert has been validated.

as a suspect. After investigation, he was transferred to the Nyankunde CTE. No alerts were reported in North Kivu, South Kivu, Tshopo, and Haut-Uélé.

Table 4. Status of key PoE/PoC indicators by province, as of September 13, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	96.7% (58/60)	89.4% (17/19)	55.5% (5/9)	100% (18/18)	54.5% (6/11)	88.9% (104/117)
% of functional strategic PoE/PoC	100%	100%	100%	100%	54.0%	100%
Number of people who have switched to PoE/PoC	129 849 88 717		7,181	15,704	11,132	252,583
% of travelers screened	98.0%	97.9%	98.9%	98.5%	98.0%	98.1%
% of travelers who washed their hands	98.0%	97.9%	98.9%	99.4%	98.0%	98.0%
99.1 % of travelers made aware		97.9%	100%	100.0%		98.8%
Number of alerts notified	1	0	0	0	0	1
Number of validated live alerts (suspected cases)	1	0	0	0	0	1
Number of bodies recovered today	0	0	0	0	0	0
% of suspected cases transferred to CT/CTE	100%	N / A	N / A	N / A	N / A	100%
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A	N / A	N / A
% of suspected cases investigated within 24 hours	100%	N / A	N / A	N / A	N / A	100%

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in Haut-Uélé (54.5%, 6/11) and Sud-Kivu (55.5%, 5/9); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri) and continuation of the strike for non-payment of service providers of three PoCs (Nzebi, Galay and Base Yedi); low encoding of alerts in DHIS2 Tracker in Ituri; in Tshopo, 18 PoCs activated out of the 30 planned; in Sud-Kivu, absence of isolation device at the PoEs of Kavimvira, Kalundu Port SNCC and Baraka.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 44 new positive results** (26 living and 18 deaths) out of 211 new samples received and analyzed (**positivity: 20.9%**).

- **North Kivu: 12 new positive results** (7 living and 5 dead) out of 164 samples analyzed (**positivity of 7.3%**).
- **Haut-Uélé: 2 new positive results** (2 live) out of 9 samples analyzed (**positivity: 22.2%**).
- **Tshopo: 3 samples received and tested** (3 live), **all were found to be negative.**
- **South Ubangi: 3 samples collected, 2 of which are sent to Kinshasa (INRB).**

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 25 of the 61 planned septic tanks were opened (41.0%), 94 of the 100 identified households were decontaminated (94.0%), and 10 of the 12 health facilities were decontaminated (83.3%); 15 hygiene kits were distributed in Mongbwalu. Regarding the health surveillance system (HSS), 68 alerts were recorded (9 of which were postponed), 65 HSS tests were carried out out of 77 (84.4%), and 54 of the 65 bodies were swabed (83.1%).
- **In North Kivu**, 29 of the 44 planned infection control rings were opened (65.9%), 11 of the 49 identified households were decontaminated (22.4%), and 23 of the 31 health facilities were decontaminated (74.2%). No infection prevention and control (IPC) kits were distributed to households of cases, and 4 scorecard assessments were conducted with an average score of 31.6% (3 in Musienene and 1 in Kalunguta). 749 service providers were briefed, 162 community health workers (CHWs) were monitored and supported, 112 handwashing stations were monitored, 7 triage units were installed, and 240 liters of water were supplied to Oicha. Regarding blood and health surveillance (BHS), 56 alerts were recorded, 53 BHSs were conducted, and 51 bodies were swabed.
- **In Tshopo**, decontamination of the aircraft that transported a suspected case from Buta (Bas-Uélé) and initial infection prevention and control (IPC) assessment by the Meteorological Treatment Center (90.6%); monitoring and support of 13 community health workers in 5 health zones with briefing of 142 providers on standard precautions and monitoring of 11 handwashing stations. Regarding the emergency response teams (ERT), 3 alerts were recorded, 3 EDTs were carried out, and 3 bodies were swabed (100%) at the EDT.
- **In Haut-Uélé**, 2 rings were opened out of the 5 expected (40.0%), 5 households decontaminated out of 10 identified (50.0%) and 1 FOSA decontaminated out of 1 (100%), in the ZS of Isiro and Wamba; As for the EDS, 2 alerts were recorded, 1 EDS carried out and 1 body swabed.
- **In Bas-Uélé**, 1 ring was opened out of 1 expected (100%) in Ganga; 1 EDS alert recorded and 1 swabed body.
- **South Ubangi**, Disinfection of the premises of the ESS where the suspected cases had been; deployment of some PCI inputs by the EIR towards the Bulu ZS.

1.4.2. Challenges

- **In Ituri** : low opening of rings (41.0%; 25/61); insufficient decontamination teams and lack of mobility in several health zones; insufficient EDS inputs; 9 bodies not swabed, in Mandima due to lack of inputs and teams and elsewhere due to refusal by families; community resistance in Bambu, Komanda, Nia-Nia and Mangala.
- **In North Kivu** : low household decontamination (22.4%; 11/49); lack of traceability of some eligible case households; lack of mobility for the deployment of provincial IPC teams; need to activate, train and motivate decontamination teams in the uncovered health zones (Katwa, Biena, Masereka, Kyondo, Mabalako and Vuhovi).
- **In Tshopo** : insufficient IPC inputs in households, schools, markets and churches; low proportion of SSEs with an IPC score above 80% (2.2%; 2/92) and incinerator of

The Makiso-Kisangani General Referral Hospital is still out of service. **In Haut-Uélé** : lack of community IPC kits, absence of triage units in health centers, and poor timeliness of daily data from health zones.

- **In Bas-Uélé** : absence of compliant waste management areas and sorting units in the 20 ESS hotspots of 5 health zones, insufficient bins and handwashing facilities, untrained IPC providers in the 5 priority health zones (Buta, Ganga, Viadana, Titule and Poko) and absence of IPC supervisors.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, 86 new admissions were recorded in the various dedicated facilities, compared to 88 discharges, including 12 recoveries. Thus, 472 patients are hospitalized out of 1015 beds, representing an occupancy rate of 46.5%; 434/461 (94%) patients received hot meals; 10/10 (100%) children are receiving LNPE/SLM at the CTE, 4/4 (100%) at the nursery, and 29/29 (100%) in the community.
- **In North Kivu**, 46 new admissions were recorded against 53 discharges, including 2 recoveries; 332 patients are being treated in hospital, representing an overall occupancy rate of oversaturation (145.6%); 7/7 accompanying persons and 42/42 frontline providers received hot meals.
- **In Haut-Uélé**, 7 new admissions were recorded against 3 discharges. Thus, 69 patients are being treated in hospital out of 134 beds, representing an occupancy rate of 51.5%; in addition, 82 accompanying persons and 34 PPLs received three hot meals, with the exception of the Rungu and Gombari health zones.
- **In Bas-Uélé**, one (1) new admission was recorded against one (1) discharge (non-case); 7 patients remain in isolation, including 1 confirmed case taken care of at the HGR of Ganga; 9 high risk contacts are under prophylaxis in Buta and Ganga, 10 contacts having left unharmed from the follow-up in Buta.
- **In Tshopo**, no new admissions were recorded against one discharge (non-case). Eight (8) patients are in isolation out of 31 beds, representing an occupancy rate of 25.8%.
- **In South Kivu**, there were no new admissions. Seventeen (17) patients are in isolation out of 25 beds, representing an occupancy rate of 68.0%.

1.5.2. Challenges

Treatment is still partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases: Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 45% of hospitalized patients are treated outside of ETCs, and no beds are available in Butembo and Katwa. **In Tshopo**, there is a lack of safe blood and limited technical resources. **In Bas-Uélé**, the absence of CTE/CT and ambulance limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 2,307 people have been vaccinated to date out of 11,703 targeted TPL/PPL (19.7%), including 1,448 in Makiso-Kisangani, 313 in Mangobo, 312 in Kabondo, 150 in Tshopo and 84 in Lubunga.
- **In Bas-Uélé**, 500 doses of ERVEBO vaccine were received at the PEV (Population Evangelical Initiative) office in Buta, and the team responsible for issuing electronic vaccination cards arrived. In Ituri, preparations continue for the vaccination of people living in poverty, scheduled to begin on September 15, 2026, in Bunia.

1.6.2. Challenges

Low uptake of vaccination among the population and insufficient management tools **in Tshopo**, where the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, there is no microplan for

vaccination of PPL in the six priority health zones (Aketi, Ango, Bili, Buta, Ganga and Viadana).

1.7. Continuity of Care

- **In Tshopo**, 128 cases received experimental treatment, including 16 confirmed and 112 under post-exposure prophylaxis (72 treatments completed, 11 interrupted and 29 ongoing); a fourth PPL was confirmed, evaluation of CS Betshaida and development of the recovery plan for CS Fedi.
- **In Haut-Uélé**, the briefing of service providers on the specific treatment and prophylactic for Ebola virus disease.
- **In Bas-Uélé**, follow-up of contacts who have adhered to compassionate prophylaxis in Buta and Dingila and post-exposure prophylaxis of PPL high-risk contacts in the Ganga health zone.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 1,179 home visits reached 13,969 people and 581 educational talks reached 7,310 people; 15 mass awareness campaigns reached 2,888 people, 15 community debates reached 266 people and 1 advocacy campaign reached 4 people; 154 community alerts were notified (including 42 deaths), 29 of the 35 feedbacks were resolved within 48 hours and 6 of the 8 community incidents were resolved in Nyankunde, Komanda, Bunia and Nizi, for a completeness of 15 of the 28 health zones.
 - **In North Kivu**, 83,328 people were reached during 19,231 home visits, 1,688 people were reached during 62 educational talks and 5,616 during 19 mass communication actions; 320 community alerts notified out of 450 expected (71.1%), 10 feedbacks resolved out of 14 (71.4%), 1 incident resolved out of 1 (100%) and 700 people engaged out of 4,750 (14.7%); community dialogue bringing together 40 community groups in the Aloya AS (Mabalako Health Zone), working session with religious and associative leaders in Manguredjipa and prevention message launched with the support of MONUSCO in Rwangoma (Beni Health Zone).
 - **In Haut-Uélé**, 25 educational talks reached 172 people, 8 mass activities reached 3,589 people in churches, and 1 community dialogue reached 218 people, for a total of 3,979 people engaged; 7 alerts of suspected cases and deaths were reported by community actors in Isiro (2), Boma Mangbetu (2), Wamba (2), and Watsa (1); 20 feedback issues were resolved within 48 hours in Isiro; and preparations for the discharge of 17 survivors of Ebola are underway.
- continues in Pawa, Wamba and Gombari.
- **In Tshopo**, 28 mass awareness campaigns in religious settings reached 27,109 people in the 5 health zones of Kisangani; 4 educational talks reached 74 people in Bafwasende and Wanie-Rukula; and 476 home visits reached 2,707 people; 2 interactive programs and 8 public service announcements were broadcast in 4 media outlets; 3 community feedback requests were collected and all resolved within 48 hours. In Bas-Uélé, briefings and awareness sessions were held for 31 community health workers in Buta; a conference-debate on Ebola virus disease (EVD) was held at the conclusion of a retreat for priests of the Diocese of Buta (20 priests and the bishop); a poster campaign was conducted at the Ligilibe mine (Amadi health district, Poko health zone) reaching over 300 people; and a community radio station was established in Buta.

1.8.2. Challenges

Persistent community resistance: **In Tshopo**, the need for handwashing facilities in schools and rumors of a \$300 reward for reporting cases or deaths fueled infodemics that were resolved within 48 hours. A confirmed case who escaped from the treatment center remains at home, and the local health hub is experiencing a shortage of posters and banners. **In Ituri**, there is insufficient transportation to support other health hubs, a broken printer, and no internet connection for the local health hub. **In Haut-Uélé**, there is a complete breakdown of visibility tools and communication equipment in all affected health zones. **In Bas-Uélé**, roads to some health zones and areas are impassable, there is a lack of transportation and phone credit, demotivation among field workers, and poor reporting from health zones.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 20 people among the new confirmed cases and 114 of the 199 suspects under follow-up (57%) benefited from SMSPS interventions, as well as 32 of the 85 new suspects of the day (38%); 50 announcements of results made including 20 positive, 56 companions and 30 PPL supported, 27 children followed in the community, 7/7 children followed in nursery with 1 new admission, 11 of the 18 former recoveries followed in the community and 6 of the 12 recoveries reintegrated; 10 of the 28 health zones covered.
- **In North Kivu**, 38 people among the new confirmed cases and 187 confirmed cases under follow-up were taken care of, as well as 154 new suspected cases and 206 suspected cases under follow-up; 126 results announced, including 65 positive cases, 56 PPLs supported, 66 households supported, 11 of the 12 cured, i.e. 91.7% followed up; 36/36 children followed up in the community and 1/1 in daycare; 507 people (156 men, 273 women and 78 children) benefited from psychoeducation sessions and the 9 notified health zones are covered.
- **In Tshopo**, 2 confirmed cases under follow-up, 1 new suspected case and 7 suspected cases under follow-up have benefited from SMSPS interventions; 3 laboratory results announced, including 2 positive ones, 10 accompanying persons at the CTE, 2 PPL and 7 members of households and the community supported, 5 separated children followed in community and psychoeducation of households in the Makiso-Kisangani health zone; 2 recovered have joined the response teams.

In Haut-Uélé, 6 confirmed cases from the previous day and 43 confirmed cases under observation, along with 12 new suspected cases and 15 suspected cases under observation, received SMSPS interventions. Seventy caregivers at the Ebola Treatment Center (ETC) and 35 people with HIV/AIDS (PPL) received support. Fifty-one laboratory results were announced, including 20 positive results. Seven separated children received community support, and 20 household and community members received assistance. In Bas-Uélé, support for the confirmed case in Ganga, his family, and the 27 identified contacts remains limited by the lack of a psychologist in the health zone.

1.9.2. Challenges

- **In Ituri**, SMSPS interventions have limited coverage, with only 10 of the 28 health zones being covered. **In North Kivu**, interventions around the latest confirmed cases are limited in the health zones of Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi; there is a lack of childcare facilities in 10 affected health zones, and there are insufficient human resources and motivation. **In Haut-Uélé**, there is a lack of computer equipment for the pillar; mobility difficulties for community health workers in Wamba and Boma Mangbetu; and staff demotivation due to non-payment. **In Tshopo**, there are no SMSPS providers in Wanie-Rukula, Bafwasende, and Isangi, and partner participation is low. **In Bas-Uélé**, there is no psychologist in the Ganga health zone.

1.10. Logistics

1.10.1. Main actions

- **In Bas-Uélé**: guaranteed mobility for the rapid intervention team deployed in the ZS of Ganga and for the health zone teams responsible for verifying alerts and active search; the available chlorine stock is 50 kg.
- **In South Ubangi**: delivery of some PCI inputs and monitoring tools for the Bulu Health Zone; deployment of an EIR.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, the pillar provided support and security coverage for EDS teams during the swab of a deceased person to the Ndindi morgue and for the burial of two

confirmed cases as well as PCI teams for decontamination in the health areas of Boikene, Mangote and Bundji, with 100% coverage of requested interventions.

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team, a lack of financial incentives for agents and non-payment of police officers and security agents assigned to guard the ESS; lack of support from security elements in several PoCs along the Butembo axis (Kangote, Kyaghala, Mususa and Komba); delayed payment of actors involved in the response and non-compliance with commitments by some partners in several health zones.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri,

5 response staff members (5 men) received the PSEA briefing and 5 response personnel were sensitized; one partner (IRC) reported on its PSEA activities and 13 of the 36 health zones (36.1%) are aware of the reporting mechanism. In Bas-Uélé, 31 community health workers, including 18 women, participated in a briefing on sexual exploitation and abuse, in support of the CREC pillar.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- A positive signal has been given in 7 health zones in the provinces of Ituri (Adja, Kambala and Mahagi), North Kivu (Goma), South Kivu (Miti-Murhesa) and Tshopo (Lubunga and Wanierukula) which have totaled more or less **42 days** since the last notification of confirmed cases.
- Performance remains consolidated in epidemiological investigations (**100% of suspected cases investigated**).
- In the provinces of Ituri, North Kivu and Haut-Uélé, sustained transmission is noted while the provinces of Tshopo and Bas Uélé which show sporadic notification.
- In the last 24 hours, **58 new confirmed cases**, including **23 community deaths and 12 intra-CTE deaths**, have been reported, bringing the national total to **7,258 confirmed cases and 3,510 deaths (case fatality rate: 48.4%)**.
- Ituri remains the epicenter of the epidemic, accounting for **78.0%** of confirmed cases and **75.9%** of new cases reported in the last 24 hours.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.
- Strengthen preparedness and response to Ebola virus disease in the provinces of the river corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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